

Long-term strat & product vision

June 2026

This is a working document. First part is 90% human-written, Appendices are agent-written. Formatting is obviously AI.

Why we exist

I want to start with the why, because the strategy will change but the reason we are doing this should be steadier. The mission, so far: **clarify and connect care journeys, for you and your loved ones.**

This rests on the belief we started from. The patient is massively underserved, and the patient is the unlock for a healthcare system that is otherwise stuck.

Scribes took off because the technology finally caught up, but the real gap was never the doctor's note. It was the person walking out of the room with a failing memory, a notepad, and a letter full of Latin that tells them they have cancer. EHRs have not fixed that: they gave us clunky software, fragmentation, and portals that store data without ever explaining it. AI assistants are individualistic yet generic, and omnipotent yet unreliable.

Summaries cannot be the goal

Summary tools are quickly becoming a commodity, and they keep getting more reliable. On the business side they are becoming standard for every conversation.

Why that is not enough

- Better summaries are only defensible if you have something no one else has: proprietary models, unique validation data, or a sharp niche. We do not have this.
- EHRs and scribes are catching up. Within about a year, patient summaries are native. When ChatGPT or Gemini ship dedicated conversation summaries, that attacks us directly. Saying "doctors will never allow it" is wishful thinking.
- A summary is relevant around the appointment, and maybe the day after. Even when you are severely ill, that is a reason to open Ditto weekly at most. We remain a convenient feature, not something people don't want to live without.
- Low frequency is hard to monetize. Consumers do not pay easily for health apps, and insurers want activity before they reimburse.

- Pushing frequency or growth features for their own sake is a dead end. Unless we solve real problems, we can only nudge so far in the direction we want.

The good part

- Our summaries are genuinely better than most alternatives so far, and people trust them.
- We are roughly 100x better than the alternative, which is forgetting.
- Summaries and recordings hold far more potential than we unlock today.
- The summary is the way in. We have to squeeze the most value out of that episodic moment.

From there the list is long: auto-update your health profile, nudge and support follow-up actions, keep the family informed dynamically, link you to peers, explain your treatment in podcast or visual form, point you to a second opinion, find a fitting trial. The summary is the door, not the room.

The long-term: six product end-states

We have been circling various end-states and mix-and-match them often, I feel. I would like to name them and I have also scored them on six dimensions; the scores live in Appendix B.

Two labels run through all of this. **Core** is the intelligence we build the company on, from understanding to advising. **Additional** is everything that strengthens or extends the Core but is not the foundation: a record we integrate, a coordination or social layer we lean into selectively, an operational front door.

1. Care Companion (Core, emphasis today)

Relation to the vision: the base. Understanding your own care, the job Ditto already does. Everything else stacks on this.

What the product does: clarifications, an overview and visualisation of your care, disease-specific context, context-aware AI Q&A, symptom-tracking, potential journaling. Significant AI potential.

Strengths: better than general-purpose AI on your actual care; trusted; EU-native; oncology depth; it brings AI to people who do not reach for it elsewhere.

Risks: the summary artifact itself commoditises (Appendix C). Can move into MDR. Leans into broader consumer health trend.

2. Trusted Health Assistant (Core)

Relation to the vision: the top of the intelligence ladder, what the companion becomes as you keep adding intelligence.

What the product does: proactive, personalised advice and nudges on the medical behaviour you can control; personalized reminders; wearable inputs; a full AI assistant. Very AI heavy.

Strengths: the highest technology IP; once trusted, strong lock-in; enormous monetization potential if winning.

Risks: Individualised advice triggers MDR Class IIa and AI-Act high-risk, and you compete with consumer big tech. Most risk of friction with doctors.

3. Patient-Owned Record

Relation to the vision: let patient own all their medical data. Probably not a destination on its own. It is the input that lets the Companion and the Assistant see the whole picture.

What the product does: ingest all available medical data, make it understandable and shareable; data integrations; a possible EHDS launching partner.

Strengths: if it were the superior EU solution, it would carry immense lock-in.

Risks: data alone is low value without insight; you compete with governments and EHRs; heavy dependency on integrations that can be cut; owning the record pays close to nothing (Appendix D).

4. Care Coordination

Relation to the vision: improves how families manage the disease together. It rides the Companion, but makes it multi-player and operational.

What the product does: factual medical updates, a shared overview, agenda and task management, flexible managed accounts, support requests. Probably low AI.

Strengths: increasingly needed as care gets complex and asking more of caregivers; more focused and tailored than WhatsApp group; high practical value.

Risks: hard to make fly without institutional integration; monetization probably quite bounded; depends on the whole group, professionals included, adopting it (cf. Caren).

5. Social Care Platform

Relation to the vision: improves the experience of not being alone. Connection that is enabled by better understanding.

What the product does: support gestures, shared daily updates, narrative or blogging, finding peers, communities.

Strengths: network effects; it could become the go-to place to share anything health-related.

Risks: competes with WhatsApp and existing social platforms; improves perception, maybe not outcome; super hard to monetize

6. The Front Door of European healthcare (Core)

Relation to the vision: the “do-it-all” entry point. Super broad, category-defining, every patient-centric.

What the product does: scheduling, planning appointments, second opinions, finding your data, messaging professionals, prescriptions; a lot of care-companion-ship too probably and potentially even care coordination.

Strengths: a closed provider moat is genuinely defensible; clear B2B monetization (the Doctolib model); high frequency for whoever owns it; can be anything

Risks: owning it with monetization probably means becoming a provider-SaaS company. The lighter move is to plug into existing infra and own the intelligence on top; so broad it does not guide us effectively

Reflection on board meeting input in this regard

- We brainstormed on targets and what Ditto should be. “The trusted place for your healthcare”, “the go-to digital solution half of cancer-patients use”. I like these statements, but from a product perspective they don’t give me the guidance I need to make better decisions. They map to any of these states.
- More narrow focus on oncology *does* help: creating focus, reducing optionality, increasing relevance, easing validation.

The synthesis

Personally, I believe that intelligence should be the Core. It runs from understanding (Care Companion, today) to thinking along (Trusted Health Assistant). It grows into two directions: being more intelligent in certain moments (smarter summaries), and over time, by moving into the day-to-day and weekly needs between appointments, where frequency lives.

The Patient-Owned Record is not a goal in itself, it is a multiplier: if we can integrate it, the assistant finally sees the full picture. But, if we don’t push on integration we are probably late to the party. Currently, I think today we capture a fraction of your health information and without integrations, we rely on manual effort and that is a dead-end.

The Front Door includes a lot of operational use cases: scheduling, prescriptions, messaging, data access. It is where Doctolib and the portals live. It can stack on top of the Care Companion, since it brings reach and data, and being the front door is itself defensible. But not if you’re a side-entrance, little people know. Currently, I don’t think we follow a strategy that competes with Doctolib on their turf (probably a good thing though, for now!).

Coordination and social for me feel like add-ons and not the core product proposition, but here we might think differently within the team. I propose that we take the parts that make the companion a better experience, and we experiment where there is a real traction.

Bringing it back to three product dimensions

This section pulls the end-states above together. They are not separate products but facets of one vision: Ditto as the place you turn to make sense of your care and get through it well, for you and the people around you. Combined, they collapse into three directions the product can grow along. They overlap by design, and each one is simply a different centre of gravity for the same promise: take the confusion out of healthcare and put something intelligent, and human, in its place.

1. The intelligent care companion

The patient is the centre. We make you understand what is happening, then think along with you: tracking, advising, and helping you recover or get through treatment as well as possible. Others can be pulled in, but comprehension that turns into intelligence is the job. (Care Companion growing into the Trusted Health Assistant, fuelled by the Patient-Owned Record.)

First phase:

- Consult summary
- Trusted information and resources, matched to your situation
- AI question-answering grounded in your care
- Symptom overview with light correlations
- Shareable health updates (lightweight)
- Medical diary
- Visit preparation: what changed, the questions worth asking

Deeper:

- Proactive, personalised care advice on what you can control
- Symptom triage
- Wearable integration, read against your appointments and medications
- Lab and result interpretation, tracked over time
- Medication intelligence: what to expect, interactions, adherence
- Light accountability: did the advised follow-up actually happen
- Treatment explained in audio or visual form
- Second-opinion and clinical-trial pathways
- A self-updating health profile

2. The social care platform

Turns Ditto from single-player into multi-player. Two sides. (Care Coordination plus Social Care Platform.)

More care coordination:

- Shared, factual overview of what is going on
- Agenda, task management, and task allocation
- Two-way messaging integrated with professional systems
- Managed accounts
- Close circle and wider circle
- Family accounts with shared profiles (for children)
- Light, frequent care updates

More social platform:

- Peer finding
- Community building
- Public sharing of health updates

3. The EU healthcare navigation platform

The front door of European healthcare: the place you open to do anything in care. (The Front Door, on the Patient-Owned Record's data layer.)

- Scheduling and appointments
- Referral managing
- Planning second opinions
- Medication ordering and refills
- Access to all your medical data
- Reaching and messaging your professionals
- European Health Data Space integration
- Patient-initiated data sharing with providers who don't hold your records
- An always-current medication overview

Based on this, we can map all potential features and overlap also in a table:

Category	Feature	Why it matters	1. Intelligent care companion	2. Social care platform	3. EU healthcare navigation
Understanding	Consult summary	Visit turned into plain language	Essential	Adjacent	Nice to have
	Trusted info & resources	Reliable answers, not the open web	Essential	Nice to have	—
	Treatment explained (audio/visual)	Comprehension in a format that lands	Essential	Nice to have	—
	Visit preparation	Walk in knowing what to ask	Essential	Nice to have	Adjacent
	Medical diary	Journal your own experience	Essential	Adjacent	—
Intelligence	AI question-answering	Instant answers grounded in your care	Essential	Nice to have	Nice to have
	Symptom overview + correlations	See patterns over time	Essential	Adjacent	—
	Proactive care advice	Guidance on what you can control	Essential	Nice to have	—
	Symptom triage	What to flag, when to call	Essential	Adjacent	Nice to have
	Lab & result interpretation	Understand values and trends	Essential	Nice to have	Adjacent
	Medication intelligence	Effects, interactions, adherence	Essential	Adjacent	Adjacent
	Light accountability	Did the follow-up happen	Essential	Adjacent	Nice to have
	Second-opinion & trial pathways	Routes to better options	Essential	Nice to have	Adjacent
	Self-updating health profile	Living record, no manual effort	Essential	Adjacent	Adjacent
Sharing	Shareable health updates	Broadcast once, not ten times	Essential	Essential	—
	Light, frequent care updates	Keep the circle gently informed	Adjacent	Essential	—
	Public sharing of updates	Share the journey openly	Nice to have	Essential	—
	Peer finding	Connect with similar patients	Nice to have	Essential	—
	Community building	Support, and not alone	Nice to have	Essential	—
Management	Shared, factual overview	One source of truth for the circle	Adjacent	Essential	Nice to have
	Agenda, tasks & allocation	Divide the caregiving load	Nice to have	Essential	Adjacent
	Managed accounts	Act for someone who can't	Adjacent	Essential	Adjacent
	Close & wider circle tiers	Control who sees what	Adjacent	Essential	—
	Family accounts & shared profiles	Manage care for children	Adjacent	Essential	Nice to have
	Scheduling & appointments	Book and manage visits	Nice to have	Adjacent	Essential
	Referral management	Track and route referrals	—	Nice to have	Essential
	Planning second opinions	Organise a second view	Adjacent	Nice to have	Essential
	Medication ordering & refills	Get medication without friction	Nice to have	Adjacent	Essential
Integrations	Wearable integration	Signals in the context of care	Essential	Nice to have	Adjacent
	Messaging professional systems	Two-way contact with care team	Nice to have	Essential	Essential
	Access to all medical data	Your records in one place	Adjacent	Nice to have	Essential
	EHDS integration	Pull records EU-wide	Adjacent	—	Essential
	Patient-initiated data sharing	Share records with new providers	Nice to have	Adjacent	Essential
	Always-current medication overview	Single source for your medication	Adjacent	Adjacent	Essential

Two ways of reasoning

Above is a way of reasoning backward for how we develop the product:

Backwards from a vision. Imagine what Ditto should look like in a few years and trace steps backward on what we build now. This keeps us coherent and stops us becoming a pile of incoherent features. We do not know the precise route, but we have directionality.

If this end-state vision is grand and contrarian and overlooked, it might just be the right direction. But it also risks reasoning towards something that would be great, but is not rooted in reality. So we can also reason like:

Forwards from jobs to be done. Watch how people actually struggle in healthcare today and solve one job, then the next, so Ditto solves more and more real problems over time. The care-management jobs people already struggle with are clear: booking and landing appointments, finding their own data, looking things up online, scrappy notepad, half-informed loved ones. This way of reasoning forward ensures we are useful and grounded in how people already behave.

Monetization

The uncomfortable truth is that almost nothing in this space monetizes on patient-pay directly. Fitness is easy. Wellness is doable. Longevity is easy. Healthcare is hard. The honest answer to “can a reimbursed app be a unicorn?” is probably “no”. The reimbursed market is relatively small and has bankrupted pioneers; the companies that got big did it on B2B at scale, see Appendix D. For consumer-side monetization, you need volumes and frequent value. Or if you want to sell-eyeballs via ads you need crazy traffic. So reimbursement is a credible starting point, but the monetization engine probably requires B2B, I think. This is a decision-axis to evaluate certain product visions.

The how for product

We need and can move faster. To do so, what will help:

- **Creating more clarity** on where we want to be in a few years helps prioritize product in the short term. E.g. if we want to be an intelligent assistant, we should favour interactive AI chat over task management for care circles.
- Not committing to a direction keeps options open, but it can just as easily lead to a frankenstein product and to windows closing. If we invest heavily in data integrations and anticipating EHDS, we slow down on adding real intelligence to the data users already put into Ditto. That is a genuine trade-off, not a free choice.
- We should make **bigger bets** in shorter cycles and learn fast. Anthropic spends roughly 70% of its time on big bets; the rest is polish, maintenance, and low-hanging fruit.

- If the strategic priorities are clear, we can seriously consider **buy** and **acquisition options** too, to directly buy distribution, scale, and so on.
- We should be careful with the framing of “what would be good for Ditto.” On the product side, we should address actual (or unaware) problems and needs.

Core question

1. **Why do we exist?**
2. **What does Ditto solve or improve?**
3. **Who are we building for, primarily?**
4. **What is, or do we want to be, our moat?**
5. **How do we monetize it?**

Appendices: the evidence base

Sourced from two AI-research programs. Full AI writing, but based on human input.

Appendix A. Market forces: a heating market is a tailwind

For years, patient-facing health tech was a backwater. In the last roughly eighteen months it became one of the hottest markets in technology. Capital is pouring in, the largest companies in the world are all building here, and even direct analogs of Ditto are now funded. The category we bet on is being validated in real time. That is a tailwind, not only a threat.

Where the capital and the giants are going:

Player	Move	Signal
Whoop	\$575M at a \$10.1B valuation; ~2.5M members, ~\$1.1B run-rate. Strategic investors Abbott and Mayo Clinic; SHA Wellness clinical tie-in.	Consumers pay for health at scale, and the consumer winner is already going B2B and clinical.
Oura	\$900M+ at ~\$11B; 5.5M+ rings, >\$1B 2025 revenue. Essence Healthcare (Medicare Advantage) gives members free rings; US DoD partnership; Dexcom invested \$75M.	Same pattern: a consumer hardware hit moving into payer and institutional channels.
Apple	Watch FDA-cleared for hypertension and sleep apnea, but scaled back its "AI doctor" (Project Mulberry, Feb 2026).	Even Apple could not ship the "explain my health" layer. It is genuinely hard.
Google	Gemini "Personal Health Coach" became Google Health Premium (\$9.99/mo) after 500k+ testers.	A big-tech consumer health subscription, live and paid.
OpenAI / Anthropic / Google	ChatGPT Health (7 Jan), Claude for Healthcare + HealthEx (11 Jan), Gemini Health Coach, all within one week, Jan 2026.	The three biggest AI labs entered consumer health at once, aimed at "explain my records."
Epic	"Ask Emmie" patient assistant; 85% of customers live with genAI; already live in NL hospitals.	The incumbent EHR is moving patient-direct.
Kin Health	\$9M (May 2026), US-only; the closest direct analog, same recorded-visit wedge plus care-circle loop.	Even our exact shape is now venture-funded. The window is real but watched.

The consumer winners are themselves going B2B. Whoop sits next to Abbott and Mayo Clinic; Oura is in Medicare Advantage and the DoD. This both shows the market maturing and foreshadows Appendix D: even the consumer hits make their durable money through institutions.

The demand behind it is exploding, not hypothetical. Around 40M people ask ChatGPT a health question every day (230M+ per week). Consumer AI-for-health use doubled in a year (16% to 32% in the US; the EU runs twelve to twenty-four months behind). Patient comfort with AI listening in the room jumped from 21% (2024) to 60% (2026). Voice is becoming the default interface. Every one of those curves points at the exact moment Ditto already owns: a spoken medical conversation people want to understand.

Why a hot market is good for us, not only scary:

- **It validates the category.** We are no longer evangelising a niche. The biggest companies in the world are telling the market this matters.
- **The tailwinds are on-thesis.** Voice-first, ambient capture, agentic assistants, and rising consumer comfort all favour a product whose raw input is the recorded visit.
- **The giants are aimed elsewhere.** Wearables own what your body did. The labs answer generic questions and ground in your records for you. None owns what the doctor actually said, none is built for the family around a serious diagnosis, and none is EU-record-native.
- **The best-resourced player could not do the hard part.** Apple shelved its AI health coach because “explain and advise on my health” is hard. That is a signal, not a gap, for a focused player.
- **Heat brings capital and exits.** A hot, well-funded category is a better place to raise, partner, and be bought into than a cold one.

The flip side of the same heat: the generic summary commoditises fast. That is Appendix C.

Appendix B. The six end states, scored

The bodies for each end state are in the main document above. This is the quantification, the same way as the interactive decision map: 1 to 5 per dimension. Five dimensions are higher-is-better; Regulatory is a burden, so a higher score means heavier MDR and AI-Act exposure. Scores are research-informed judgement from the two market programs, for recalibration at founder alignment, not measured fact. The table is set landscape on the next page for readability.

Appendix B. The six end states, scored

End state	Frequency	Defensibility	Monetization	User value	Excitement	Regulatory	Role
Today: the bare summary	2	2	2	3	2	2	Starting point
1. Care Companion	3	4	3	4	4	2	Core
2. Trusted Health Assistant	5	4	4	5	5	5	Core
3. Patient-Owned Record	2	2	2	3	3	3	Multiplier
4. Care Coordination	3	2	2	4	2	1	Additional
5. Social Care Platform	3	2	2	3	5	1	Additional
6. Front Door	5	4	4	4	4	3	All-encompassing

Scale 1 to 5 per dimension. Excitement = company excitement. Regulatory = regulatory burden, where a higher number is heavier MDR / AI-Act exposure, so lower is better. Core rows are shaded; "Today" is the baseline

Appendix C. Conversation summaries: the commoditization clock

The patient-friendly summary is commoditizing from every direction. The institution will likely ship it for free in NL and BE around 2027 to 2028. This is the clock the whole strategy runs against.

Vector	Signal	Status and timing
Portal / EHR	ChipSoft HiX “patiëntvriendelijke brief”; ZAS Antwerp live 2026-04-16 (~40k/yr).	Patient-friendly letters already shipping in NL/BE; standard-of-care ~2027 to 2028.
Incumbent EHR	Epic “Ask Emmie.”	Patient-direct 2026 to 2027; no confirmed EU deployment yet.
Scribe	Nuance DAX / Dragon Copilot.	GA NL/BE 2026-02-12, clinician-only so far.
Scheduling incumbent	Doctolib Consultation Assistant (6M+ after-visit summaries since Oct 2024); Doctolib Parents (patient AI) early 2026.	A scaled incumbent already bundles summary plus assistant.
B2C consumer	Simply Onno (DE, 60k+ summaries).	Real traction on the comprehension job; comprehension alone is not defensible.
Direct analog (US)	Kin Health (\$9M, May 2026), Hedy, Medcorder.	Same wedge plus care-circle loop; geography protects the EU for now.

The gap that still holds: patient-initiated, cross-provider, recording-based, multilingual, and oncology-deep is still unoccupied. Nobody is building Ditto’s exact product better. The threat is commoditization, not a head-on clone yet.

Demand is not the question. Patients forget 40 to 80% of a visit (~\$236B/yr in the US), there are 63M US caregivers (NL 1.9M at parity), 76% carry at least one chronic condition, and 31 to 38% of summary-engagers already share with someone. The user, the moment, and the problem are validated. The race is the moat, not the demand.

Implication: move from a weak relationship moat to a strong one before the artifact goes free.
No

The summary is the way in, not the thing we defend.

Appendix D. Monetization: routes and realism

Monetization is a real constraint, but it is not a wall. With enough scale and trust, most things monetise. The honest questions are which route fits us, what the evidence says works and fails, and how big each route can actually get. Health is harder than its neighbours: fitness, wellness and longevity monetise on consumer payment relatively easily, because the use is habitual and aspirational. Episodic, severe healthcare is harder. So we plan realistically, lead with the most reachable route, and stop treating reimbursement as the prize.

The routes:

Route	What it is	Realism for Ditto
Home-market B2B licensing	Hospital or insurer pays per-pathway or annually for the value a better-informed patient creates: comprehension, fewer crisis contacts, channel replacement, no-show reduction. Rides the Dutch B2B channels, the diga.nl scheme, and €2.8B IZA funds.	The engine. Lead here.
Pharma-sponsored / RWD	Pharma funds patient-support, adherence, and consented real-world data. Validated patrons (Novartis, Sciensus).	Real money, near-term, but carries a trust cost. A deliberate fork, consent-first only.
Reimbursement (DiGA / diga.nl)	Clinical evidence, then a reimbursed tariff. Achievable at Class I; the wall is an RCT, not device class.	A credible starting point, but bounded. Not the engine (see below).
Consumer subscription	Patient or caregiver premium tier.	Works only with frequent, recurring value (see below). A topper, not a base, in episodic illness.
B2B2C	Serve the patient free or near-free; bill the institution that benefits.	The shape that fits a patient-first product. The synthesis of the routes above.

Consumer subscription works only when the value recurs. SkinVision (Dutch skin-cancer checks) sustains a consumer subscription largely on its own, with only light insurer top-ups (CZ, Zilveren Kruis), because the worry and the check recur. The catch: even done well, its revenue is ~\$3.8M (2025). Whoop and Oura sustain consumer-pay through a daily habit. The lesson for us: in episodic severe illness, consumer-pay only holds if we engineer recurring value (the climb up the ladder, treatment-window tracking). Without frequency, a subscription does not stick.

Reimbursement is bounded, a starting point and not the engine. Worth having for credibility and a first revenue line, but you do not build a large company on it:

- The entire German DiGA market, the biggest reimbursed digital-health market in the world, was ~€171M in 2025 across ~58 apps (~€3M each on average), after tariffs are roughly halved in negotiation (median ~€221 per 3-month prescription).
- Reimbursement alone has killed its pioneers. Pear Therapeutics, the FDA-cleared pioneer of prescription digital therapeutics, went bankrupt in 2023 and sold its assets for ~\$6M, with its CEO blaming insurance denials. A reimbursable product is not the same as a business.
- No digital-health company has become a unicorn on reimbursement revenue alone.

The companies that got big did it on B2B at scale. Hinge Health IPO'd in 2025 at ~\$3B on ~\$588M revenue (and turned profitable); Sword Health is valued at ~\$4B. Both were built on selling to self-insured employers, not per-prescription reimbursement. Doctolib scaled on provider SaaS. The pattern is consistent: the engine is B2B contracts on the value you create, at scale. One caveat for us: the US employer-benefit model does not transplant to the Dutch market, so our B2B engine is hospital and insurer licensing, plus pharma, not employer benefits.

What fails or disappoints (caution cases): consumer subscription on low-frequency illness (every D2C health-coach player pivoted to B2B); owning the record (Google Health, HealthVault, Vivy all shut, value accrues to reasoning over the record); standalone communities (PatientsLikeMe became part of UnitedHealth, monetised as data, not subscription).

The realistic read: use reimbursement (diga.nl, B2B tariffs) as a credible starting point, but build the real engine on home-market B2B licensing (hospital and insurer) and pharma, with consumer-pay as upside that only grows once we deliver recurring value. Lead B2B, serve the patient (B2B2C). No single route is the only door, but reimbursement is not the one that leads to a large company.

Therapy areas × the three directions

An updated read of the four therapy areas against the three synthesized directions. The first table profiles each area (shape on top, strategic read below the divider); the second shows which direction fits best.

Area profiles

	Oncology & hematology	Neurodegenerative	Other severe illnesses	Pregnancy
Comprehension difficulty	High. Dense jargon, multimodal; the Latin-letter problem.	High, but the caregiver understands, not the patient.	High. A severe diagnosis, a lot to absorb.	Lower. Well-defined, less jargon.
Care complexity & providers	High. Oncology (med/surgical/radiation), haematology, nurses, GP, tumour boards.	Very high, fragmented. 13+ provider types, 62 networks, case managers.	High. Specialist + GP + allied; varies by condition.	Medium. Gynae, midwife, VSV, GP; well-defined.
Involvement of loved ones	High. An active care circle that wants to help.	Very high. Caregivers are the care (~800k informal).	Medium-high, varies by condition.	High, positive. Partner and family; joyful sharing.
Emotional charge	Very high.	High, sustained over years.	High.	Medium. Anxiety, not dread.
Recording today	Growing. Our wedge, strongest traction.	Low at decline; caregiver may record.	Low-medium.	Low-medium.
Pain / need (summary)	High	High	High	Medium
Competition	Med-high, rising. Clinic ePRO (Resilience, Kaiku, Noona); consumer apps (Belong, Outcomes4Me); LLMs commoditise summaries.	Low-medium. Big AI assistants miss declining users; few EU coordination players.	Medium. Few dedicated players; big tech and LLMs apply.	High. Flo, Peanut, Ovia, midwife apps; crowded consumer.
Product change to kickstart	Low. Companion fits; depth built.	Medium. Needs a care-circle build.	Low. Mirrors oncology.	Medium. Positive-journey UX, social.
Monetization potential	High. Most spend; pharma RWD; exits.	Medium. Wlz/Wmo/Zvw; reimbursement unproven.	Med-high. Mirrors oncology; varies.	Medium. Consumer-pay tilt; weak reimbursement.

Which direction fits best

Direction	Oncology & hematology	Neurodegenerative	Other severe illnesses	Pregnancy
Intelligent care companion (1)	High	Medium	High	Low
Social care platform (2)	Medium	High	Medium	High
EU healthcare navigation (3)	Medium	Medium	Medium	Low
Best-fit direction	1. Companion. Understanding and intelligence carry the value.	2. Social. Coordinating providers and caregivers is the job.	1. Companion. The oncology playbook, broadened.	2. Social. Low complexity; value is sharing and peers.

The read

The companion leads in oncology and other severe illness; the social platform leads where the people around the patient carry the load (coordination in neurodegenerative care, sharing in pregnancy; taking care of elderly parents); navigation leads nowhere as a wedge. Pregnancy is the clear outlier: low fit with the intelligent companion, high fit with the social platform, so it is a social and growth bet rather than a clinical-depth one. Lead oncology and other severe illness on the companion, treat neurodegenerative as the coordination play with a deliberate care-circle build, and run pregnancy through the social platform.